

Surgery for small bowel cancer

Surgery for small bowel cancer involves removing all or part of the cancer with an operation. It is the main treatment for small bowel cancer.

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Small bowel cancer surgery

Adenocarcinoma is the most common type of [small bowel cancer](#) (also called small intestine cancer). Surgery is the main treatment for adenocarcinoma of the small bowel. Some people will have [chemotherapy](#) as well.

Your doctor and nurse will explain the aim of the operation before your surgery. They will answer any questions you have. To [prepare for surgery](#), you might have some checks and tests at a pre-assessment clinic.

Your doctors will explain the aim of the operation to you before your surgery. They will try to answer any questions you have.

Sometimes the surgeon may have to make decisions during surgery. This is because they might find out more about the size and position of the cancer during the operation. If they think this might happen, they will talk to you before the operation about the possible outcomes.

The type of surgery you have depends on:

- the size and position of the cancer
- whether it has spread to any surrounding areas
- your general health.

Removing the cancer

You may have surgery to remove the part of the [small bowel](#) where the cancer is. The 2 open ends of the bowel are then joined together. The join is called an anastomosis. This type of surgery is also used if the bowel is blocked.

If the cancer has not spread to other tissues or organs, the surgeon will usually remove all the cancer, some surrounding healthy tissue and nearby lymph nodes.

The surgeon may need to remove the gallbladder and part or all of the pancreas, if the cancer:

- is in the top of the duodenum
- has spread to nearby tissues or organs

Depending on the position of the cancer, your surgeon may need to remove part of your colon too.

Your surgeon will explain what type of [surgery](#) will be best for you.

Stomas

Sometimes the bowel cannot be joined back together during the operation. In this situation, the surgeon may make an opening in the abdominal (tummy) wall during the operation. They then join the end of the bowel to the surface of the abdomen. This is called a [stoma](#). You may have a stoma to give the small bowel time to heal. Your surgeon will be able to tell you if you need a stoma.

A stoma is round or oval, and it looks pink and moist. It has no nerve supply, so it does not hurt. A stoma made using the small bowel is called an ileostomy. This is because it is made using the lower part of the [small bowel](#), called the ileum.

If you have a stoma, stools (poo) will no longer pass out of the rectum (back passage) in the usual way. Instead, they pass out of the stoma, into a disposable bag you wear over the stoma.

The stoma is usually temporary. People with a temporary stoma have a second, smaller operation a few months later. This second operation is to close the stoma and rejoin the bowel. It is called a stoma reversal.

Being told you need a stoma can be distressing. Most people find they get used to the stoma over time. The stoma care nurse at the hospital will help you look after the stoma for the first few days. They can give you support and information on caring for your stoma when you go home.

Our [cancer support specialists](#) can give you more information about having a stoma. You can call the Macmillan Support Line for free on [0808 808 0000](#) or chat to our specialists online.

You can also visit [Macmillan's Online Community](#) to talk with people who have experience living with or after a stoma, share your experience, and ask an expert your questions.

Bypass surgery

Small bowel cancer can narrow or block the small bowel. This is called a [bowel obstruction](#). It may cause tummy pain and vomiting. It needs to be treated urgently.

If it is not possible to remove the cancer, surgeons can sometimes do an operation to bypass the cancer. They do this by rerouting the small bowel around the cancer.

If you are not able to have surgery, your cancer doctor may be able to pass a small metal tube into the small bowel. This is called a stent. Once in place, the tube can expand and open up the small bowel. This can relieve symptoms caused by an obstruction. The cancer can sometimes be removed later.

After your operation

Bowel cancer surgery is a major operation. You may have to stay in intensive care or a high-dependency unit for 1 or 2 days. When you are well enough, you can move back to a general ward.

When part of the small bowel has been removed or bypassed, you may need to have a special diet, supplements or medicines. These may help you digest and absorb food. It will depend on the type of surgery you have had. Your doctor or nurse will explain this to you.

We have more information about [what to expect after surgery](#).

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our small bowel cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

AB Benson, AP Venook, MM Al-Hawary et al. Small Bowel Adenocarcinoma, Version 1.2020, NCCN Clinical Practice Guidelines in Oncology. Journal of the National Comprehensive Cancer Network, 17(9), 1109-1133. Available from: www.jnccn.org [accessed January 2023].

National Institute for Health and Care Excellence (NICE). Wireless capsule endoscopy for investigation of the small bowel. Published: 15 December 2004. Available from: www.nice.org.uk/guidance/ipg101 [accessed January 2023].

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Reviewer

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Bowel (colon and rectal) cancer forum

An anonymous network of people affected by colon, rectal, small bowel, or other colorectal cancers, which is free to join. Share experiences, ask questions and talk to people who understand.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

[Get help](#)